

smart Health Report

An Insightful Health Analytics Report
for Easier Understanding

Prepared For

Mr Ravi Tiwari

M 34



Name
Mr Ravi Tiwari

Gender
M

Patient ID
13173361

Age
34

Your Health Summary

Understand Your Health At A Glance
Your Personalized Health Summary is Now Available.

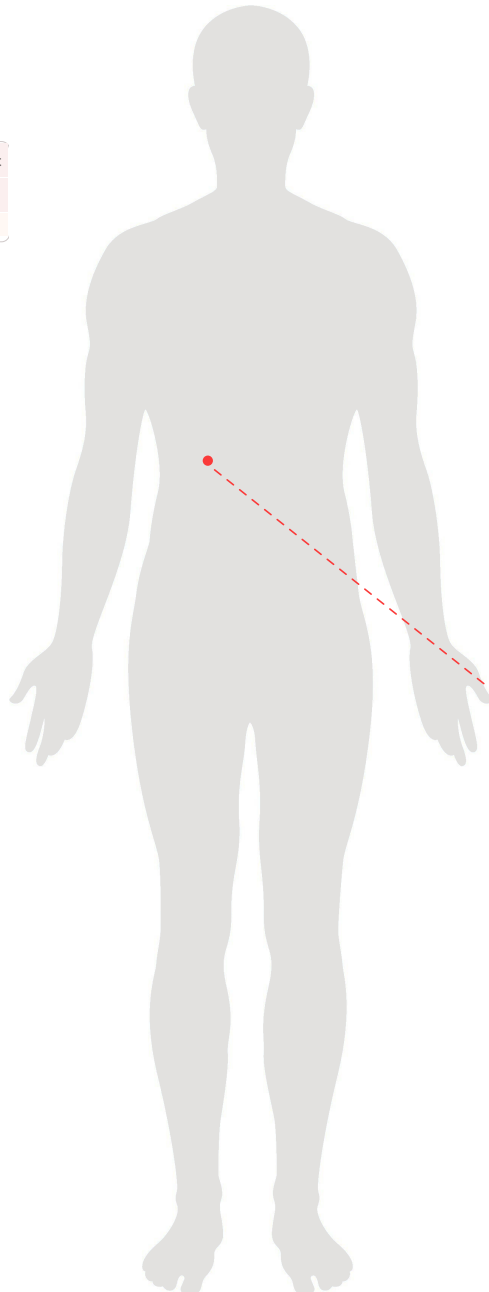
[View Detailed Summary on our App](#) 

Health Summary



BLOOD COUNTS

Test Name	Result
ESR - Erythrocyte Sedimentation Rate	32
Please Watchout	



LIVER PROFILE

Test Name	Result
SGPT/ALT	52.5
Please Watchout	



ANEMIA STUDIES

Everything looks good



Patient NAME : Mr Ravi Tiwari
 DOB/Age/Gender : 34 Y/Male
 Patient ID / UHID : 13173361/RCL8897293
 Referred BY : Self
 Sample Collected : Jul 19, 2025, 09:06 AM

Report STATUS : Final Report
 Barcode NO : 27613041
 Sample Type : Whole blood EDTA
 Report Date : Jul 19, 2025, 01:01 PM.

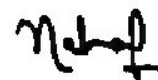


Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Fever Package Advance Plus

Complete Blood Count (CBC)

RBC Parameters			
Hemoglobin <i>Cyanide free spectrophotometry.</i>	13.6	g/dL	13.0 - 17.0
RBC Count <i>Electrical impedance</i>	4.6	$10^6/\mu\text{l}$	4.5 - 5.5
PCV <i>Calculated</i>	40.1	%	40 - 50
MCV <i>Calculated</i>	86.8	fl	83 - 101
MCH <i>Calculated</i>	29.5	pg	27 - 32
MCHC <i>Calculated</i>	34	g/dL	31.5 - 34.5
RDW (CV) <i>Calculated</i>	13.1	%	11.6 - 14.0
RDW-SD <i>Calculated</i>	28.5	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	6.8	$10^3/\mu\text{l}$	4 - 10
Differential Leucocyte Count			
Neutrophils <i>Flow-cytometry DHSS</i>	64	%	40 - 80
Lymphocytes <i>Flow-cytometry DHSS</i>	25	%	20 - 40
Monocytes <i>Flow-cytometry DHSS</i>	9	%	2 - 10
Eosinophils <i>Flow-cytometry DHSS</i>	2	%	0 - 6
Basophils <i>Flow-cytometry DHSS</i>	0	%	0 - 1
Absolute Leukocyte Counts			
Neutrophils. <i>Calculated</i>	4.35	$10^3/\mu\text{l}$	2 - 7
Lymphocytes. <i>Calculated</i>	1.7	$10^3/\mu\text{l}$	1 - 3
Monocytes. <i>Calculated</i>	0.61	$10^3/\mu\text{l}$	0.2 - 1.0
Eosinophils. <i>Calculated</i>	0.14	$10^3/\mu\text{l}$	0.02 - 0.5
Basophils.	0	$10^3/\mu\text{l}$	0.02 - 0.5



Dr. Neha Prabhakar
 MBBS, MD(Pathology)
 DMC Registration No.- 49637



Booking Centre :- Home Collection
 Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
 DOB/Age/Gender : 34 Y/Male
 Patient ID / UHID : 13173361/RCL8897293
 Referred BY : Self
 Sample Collected : Jul 19, 2025, 09:06 AM

Report STATUS : Final Report

Barcode NO : 27613041

Sample Type : Whole blood EDTA

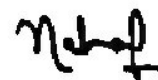
Report Date : Jul 19, 2025, 01:01 PM.



Test Description	Value(s)	Unit(s)	Reference Range
<i>Calculated</i>			
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	211	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) <i>Calculated</i>	8.7	fL	9.3 - 12.1
PCT <i>Calculated</i>	0.2	%	0.17 - 0.32
PDW <i>Calculated</i>	14.6	fL	8.3 - 25.0
P-LCR <i>Calculated</i>	25.5	%	18 - 50
P-LCC <i>Calculated</i>	54	10 ⁹ /L	44 - 140
Mentzer Index <i>Calculated</i>	18.87	%	> 13

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.



Dr. Neha Prabhakar
 MBBS, MD(Pathology)
 DMC Registration No.- 49637



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
DOB/Age/Gender : 34 Y/Male
Patient ID / UHID : 13173361/RCL8897293
Referred BY : Self
Sample Collected : Jul 19, 2025, 09:06 AM

Report STATUS : Final Report
Barcode NO : 27613041
Sample Type : Whole blood EDTA
Report Date : Jul 19, 2025, 01:31 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

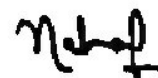
Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate MODIFIED WESTERGREN	32	mm/hr	0 - 10
---	----	-------	--------

Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and Lewis practical hematology



Dr. Neha Prabhakar
MBBS, MD(Pathology)
DMC Registration No.- 49637



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

📞 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
DOB/Age/Gender : 34 Y/Male
Patient ID / UHID : 13173361/RCL8897293
Referred BY : Self
Sample Collected : Jul 19, 2025, 09:06 AM

Report STATUS : Final Report
Barcode NO : 27613041
Sample Type : Whole blood EDTA
Report Date : Jul 19, 2025, 01:37 PM.



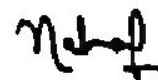
Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Malarial Parasite (MP) Smear

MP(PBF FOR MP) <i>Microscopy (Leishmans Stain)</i>	Not seen	-	Not seen
---	----------	---	----------

Interpretation:

1. Malaria is a serious parasitic diseases characterized by fever, chills, and anemia and is caused by a parasite that is transmitted human to human by the bite of infected female Anopheles mosquitoes.
2. Malarial Parasite test is performed on the blood sample to find out the level of Malaria Parasite in the blood.
3. It is conducted to conclude on Malaria and also during the treatment and after the treatment of Malaria.
4. Most people will have symptoms within 14 days of being bitten by an infected mosquito. But symptoms can show up as soon as seven days afterward or can take as long as a year to appear.
5. Clinical decision should not be based on the results of this test, but should be made by the physician after all clinical and laboratory findings have been evaluated.



Dr. Neha Prabhakar
MBBS, MD(Pathology)
DMC Registration No.- 49637



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
DOB/Age/Gender : 34 Y/Male
Patient ID / UHID : 13173361/RCL8897293
Referred BY : Self
Sample Collected : Jul 19, 2025, 09:06 AM

Report STATUS : Final Report

Barcode NO : 27613041

Sample Type : Whole blood EDTA

Report Date : Jul 19, 2025, 01:37 PM.



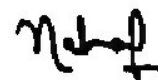
Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Malaria Antigen, Rapid Card

Plasmodium Vivax <i>Immunochromatographic</i>	Negative	-	Negative
Plasmodium falciparum <i>Immunochromatographic</i>	Negative	-	Negative

Interpretation:

Immunochromatographic Assay done for Plasmodium falciparum using Histidine-Rich Protein-II (HRP-II) and Plasmodium species (Plasmodium falciparum, P. vivax, P. ovale and P. malariae) using lactate dehydrogenase (pLDH) in human whole blood.



Dr. Neha Prabhakar
MBBS, MD(Pathology)
DMC Registration No.- 49637



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

 898-898-0606 care@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari

DOB/Age/Gender : 34 Y/Male

Patient ID / UHID : 13173361/RCL8897293

Referred BY : Self

Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 28004482

Sample Type : Serum

Report Date : Jul 19, 2025, 01:51 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Bilirubin (Total, Direct, Indirect)

Bilirubin Total <i>Diazo</i>	0.56	mg/dL	0 - 1.2
Bilirubin Direct <i>Diazo Jondrof</i>	0.24	mg/dL	0 - 0.20
Bilirubin Indirect <i>Calculated</i>	0.32	mg/dL	0.1 - 1.0

Interpretation:**Adults and children**

Increased total bilirubin that is mainly unconjugated (indirect) bilirubin may be a result of:-

1. Hemolytic or pernicious anemia
2. Transfusion reaction
3. Cirrhosis
4. A relatively common inherited condition called Gilbert syndrome, due to low levels of the enzyme that produces conjugated bilirubin.

Newborns

An elevated bilirubin level in a newborn may be temporary and resolve itself within a few days to two weeks. However, if the bilirubin level is above a critical threshold or increases rapidly, an investigation of the cause is needed so appropriate treatment can be initiated. Increased bilirubin concentrations may result from the accelerated breakdown of red blood cells due to:

1. Blood type incompatibility between the mother and her newborn
2. Certain congenital infections
3. Lack of oxygen (hypoxia)
4. Diseases that can affect the liver

In most of these conditions, only unconjugated (indirect) bilirubin is increased.

SGOT / AST

SGOT/AST <i>IFCC without P5P</i>	35.4	U/L	up to 40
-------------------------------------	------	-----	----------

Interpretation:

Serum AST is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased values are seen in liver diseases like acute viral hepatitis, cirrhosis, biliary obstruction, primary or metastatic cancer, granuloma, hepatic ischaemia.

SGPT / ALT

SGPT/ALT <i>IFCC without P5P</i>	52.5	U/L	up to 41
-------------------------------------	-------------	-----	----------

Interpretation:

Serum ALT is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased in alcoholic hepatitis, cirrhosis, hepatocellular carcinoma, chronic hepatitis. Decreased in genito-urinary tract infection, malignancy, pyridoxal phosphate deficiency states (malnutrition, pregnancy, alcoholic liver disease).



Dr. Poulami Sarkar
MBBS, MD (Biochemistry)
Consultant Biochemist
NMC Certificate No. 24-005955



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari

DOB/Age/Gender : 34 Y/Male

Patient ID / UHID : 13173361/RCL8897293

Referred BY : Self

Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 28004482

Sample Type : Serum

Report Date : Jul 19, 2025, 01:51 PM.



MC-5280

Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

C-Reactive Protein (CRP), Quantitative

CRP (Quantitative) Immunoturbidimetric	32.87	mg/L	<5
---	-------	------	----

Interpretation:**Increased CRP level:**

1. A high or increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.
 2. Suspected bacterial infection—a high CRP level can provide indication that patient has an infection.
 3. Chronic inflammatory disease—high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.
- If the CRP level is initially elevated and drops, it means that the inflammation or infection is subsiding and/or responding to treatment.



Dr. Poulami Sarkar
MBBS, MD (Biochemistry)
Consultant Biochemist
NMC Certificate No. 24-005955



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
 DOB/Age/Gender : 34 Y/Male
 Patient ID / UHID : 13173361/RCL8897293
 Referred BY : Self
 Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 28004482

Sample Type : Serum

Report Date : Jul 19, 2025, 01:36 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

WIDAL By Slide Agglutination

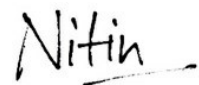
Salmonella typhi O (TO) <i>Direct Agglutination Reaction</i>	1:20	Titre	< 1:80
Salmonella typhi H (TH)	1:20	Titre	< 1:160
Salmonella paratyphi A(H)	1:20	Titre	< 1:80
Salmonella Paratyphi B(H)	1:20	Titre	< 1:80

Interpretation:**METHOD**-(Slide Agglutination)

1. Titres >1:80 of "O" antigen & >1:160 of "H" antigen for Salmonella typhi and titres >1:80 of "H" antigen for Salmonella paratyphi A & B are reactive.
2. Rising titres in paired samples taken 7-10 days apart are more significant than a single test.
3. Reactive results indicates ongoing or recent infection by Salmonella spp. and the diagnosis should be confirmed by gold standard test such as Blood culture.
4. The reactivity will vary with stage of the disease with appearance in 1st week to increase in titres till end of 4th week post which it starts decreasing.
5. In TAB vaccinated patients, high titres of H antibody of $\geq 1:160$ to each of Salmonellae is observed. They tend to persist for many months and even years while O antibody shows lower titres and disappears within 6 months.
6. Antibiotic treatment during 1st week before the appearance of antibodies tend to suppress the immune response in the form of no or decreasing antibody levels.
7. False positive results/anamnestic response may be seen in patients with past enteric infection and during unrelated fevers like Malaria, Influenzae etc. in the form of transient rise in H antibody in Widal test.
8. False negative results may be due to processing of sample collected early in the course of disease (1st week) and immunosuppression.
9. Test conducted on serum.

Uses

- To diagnose infection due to Salmonella spp. (Enteric fever).
- To monitor the progression of disease.
- To assess the response to therapy (decreasing titres) in patients being treated for Enteric fever



Dr. Nitin Arora
 DHC/R/8721
 MBBS, MD (Microbiology)



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
 DOB/Age/Gender : 34 Y/Male
 Patient ID / UHID : 13173361/RCL8897293
 Referred BY : Self
 Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 28004482

Sample Type : Serum

Report Date : Jul 19, 2025, 01:36 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Typhidot IgM, Rapid Card

TYPHI DOT/ SALMONELLA TYPHI IgM <i>Immunochromatographic</i>	Negative	-	Negative
---	----------	---	----------

Interpretation:

RESULTS	REMARKS
Positive	Indicates presence of IgM antibodies against Salmonella typhi.
Negative	Indicates absence of IgM antibodies against Salmonella typhi.

Note:

1. Its positivity in serum indicates ongoing or recent infection by Salmonella typhi and the diagnosis should be confirmed by gold standard test such as Blood culture prior to start of antibiotics.
2. IgM antibodies are typically detectable 5-7 days post symptom onset, peaking in 2nd week and frequently remain elevated for 2-4 months following infection.
3. False positive results may be due to cross reactivity with other Salmonella spp., Dengue virus infection & in patients with high levels of Rheumatoid factor.
4. False negative reaction may be due to processing of sample collected early in the course of disease, antibiotic treatment during 1st week and immunosuppression.
5. Test conducted on serum.

Use

To diagnose infection due to Salmonella typhi (Enteric fever).

Nitin

Dr. Nitin Arora
 DHC/R/8721
 MBBS, MD (Microbiology)



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
DOB/Age/Gender : 34 Y/Male
Patient ID / UHID : 13173361/RCL8897293
Referred BY : Self
Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report
Barcode NO : 28004482
Sample Type : Serum
Report Date : Jul 19, 2025, 01:12 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

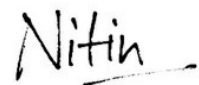
Dengue Ns1 Antigen Test, EIA

DENGUE NS1 ANTIGEN EIA	0.667	S/Co Ratio	<1 - NEGATIVE ≥ 1 - POSITIVE
---------------------------	-------	------------	---------------------------------

Interpretation:

Clinical background: Dengue virus (serotypes Den 1,2,3,4) is a flavivirus with global distribution and is transmitted by mosquitoes (Aedes aegyptii, Aedes albopictus etc). It may cause Dengue fever, Dengue haemorrhagic fever or Dengue Shock syndrome. Following the dengue infection, an incubation period of 3 to 5 days, some infections may be asymptomatic. Symptomatic patients develop fever with or without rash, severe musculoskeletal pain, headache, retro-orbital pain, petechiae etc. In most individuals there is resolution of illness without complications. In some individuals the Dengue fever may progress to Dengue haemorrhagic fever or Dengue Shock syndrome especially during repeat infection with a new Dengue Virus serotype.

Interpretation: Dengue virus antigen usually appears in blood within 24 hours of onset of symptoms to symptoms till 9 days post onset of symptoms.



Dr. Nitin Arora
DHC/R/8721
MBBS, MD (Microbiology)



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

898-898-0606

care@redcliffelabs.com

www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari
 DOB/Age/Gender : 34 Y/Male
 Patient ID / UHID : 13173361/RCL8897293
 Referred BY : Self
 Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 28004482

Sample Type : Serum

Report Date : Jul 19, 2025, 01:12 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Dengue IgM Antibodies, EIA

DENGUE FEVER ANTIBODY, IgM, SERUM EIA	0.131	S/Co Ratio	<1.0 - NEGATIVE ≥1.0 - POSITIVE
--	-------	------------	------------------------------------

Interpretation:

Clinical background: Dengue virus (serotypes Den 1,2,3,4) is a flavivirus with global distribution and is transmitted by mosquitoes (Aedes aegyptii, Aedes albopictus etc). It may cause Dengue fever, Dengue haemorrhagic fever or Dengue Shock syndrome. Following the dengue infection, an incubation period of 3 to 5 days, some infections may be asymptomatic. Symptomatic patients develop fever with or without rash, severe musculoskeletal pain, headache, retro-orbital pain, petechiae etc. In most individuals there is resolution of illness without complications. In some individuals the Dengue fever may progress to Dengue haemorrhagic fever or Dengue Shock syndrome especially during repeat infection with a new Dengue Virus serotype.

Interpretation: Dengue virus antigen usually appears in blood within 24 hours of onset of symptoms to symptoms till 9 days post onset of symptoms.

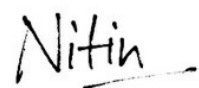
Dengue IgG Antibodies, EIA

DENGUE FEVER ANTIBODY, IgG, SERUM EIA	0.067	S/Co Ratio	<1.0 - NEGATIVE ≥1.0 - POSITIVE
--	-------	------------	------------------------------------

Interpretation:

Clinical background: Dengue virus (serotypes Den 1,2,3,4) is a flavivirus with global distribution and is transmitted by mosquitoes (Aedes aegyptii, Aedes albopictus etc). It may cause Dengue fever, Dengue haemorrhagic fever or Dengue Shock syndrome. Following the dengue infection, an incubation period of 3 to 5 days, some infections may be asymptomatic. Symptomatic patients develop fever with or without rash, severe musculoskeletal pain, headache, retro-orbital pain, petechiae etc. In most individuals there is resolution of illness without complications. In some individuals the Dengue fever may progress to Dengue haemorrhagic fever or Dengue Shock syndrome especially during repeat infection with a new Dengue Virus serotype.

Interpretation: Dengue virus antigen usually appears in blood within 24 hours of onset of symptoms to symptoms till 9 days post onset of symptoms.



Dr. Nitin Arora
 DHC/R/8721
 MBBS, MD (Microbiology)



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

 898-898-0606

 care@redcliffelabs.com

 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari

DOB/Age/Gender : 34 Y/Male

Patient ID / UHID : 13173361/RCL8897293

Referred BY : Self

Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 27916830

Sample Type : Spot Urine

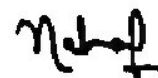
Report Date : Jul 19, 2025, 01:15 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Urine Routine and Microscopic Examination

Physical Examination			
Volume	20	mL	-
Colour	Pale yellow	-	Pale yellow
Transparency	Clear	-	Clear
Deposit	Absent	-	Absent
Chemical Examination			
Reaction (pH) <i>Double Indicator</i>	6.0	-	4.5 - 8.0
Specific Gravity <i>Ion Exchange</i>	1.025	-	1.010 - 1.030
Urine Glucose (sugar) <i>Oxidase / Peroxidase</i>	Negative	-	Negative
Urine Protein (Albumin) <i>Acid / Base Colour Exchange</i>	Negative	-	Negative
Urine Ketones (Acetone) <i>Legals Test</i>	Negative	-	Negative
Blood <i>Peroxidase Hemoglobin</i>	Negative	-	Negative
Leucocyte esterase <i>Enzymatic Reaction</i>	Negative	-	Negative
Bilirubin Urine <i>Coupling Reaction</i>	Negative	-	Negative
Nitrite <i>Griless Test</i>	Negative	-	Negative
Urobilinogen <i>Ehrlichs Test</i>	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs)	1-2	/hpf	0 - 5
Epithelial Cells	1-2	/hpf	0 - 4
Red blood Cells	Absent	/hpf	Absent
Crystals	Absent	-	Absent
Cast	Absent	-	Absent
Yeast Cells	Absent	-	Absent
Amorphous deposits	Absent	-	Absent
Bacteria	Absent	-	Absent
Protozoa	Absent	-	Absent
Interpretation: URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders. Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever			



Dr. Neha Prabhakar
MBBS, MD(Pathology)
DMC Registration No.- 49637



Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

☎ 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Mr Ravi Tiwari

DOB/Age/Gender : 34 Y/Male

Patient ID / UHID : 13173361/RCL8897293

Referred BY : Self

Sample Collected : Jul 19, 2025, 09:07 AM

Report STATUS : Final Report

Barcode NO : 27916830

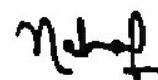
Sample Type : Spot Urine

Report Date : Jul 19, 2025, 01:15 PM.



Test Description	Value(s)	Unit(s)	Reference Range
Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.			
Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.			
Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.			
Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.			
Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.			
pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.			
Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.			
Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.			
Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.			

*** End Of Report ***

Dr. Neha Prabhakar
MBBS, MD(Pathology)
DMC Registration No.- 49637

Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., H-55, Sector-63, Noida, Uttar Pradesh - 201301

 898-898-0606 care@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Name
Mr Ravi Tiwari

Patient ID
13173361

Gender
M

Age
34

Your Health Summary

Understand Your Health At A Glance
Your Personalized Health Summary is Now Available.

[View Detailed Summary on our App](#) 

Health Advisory

 Normal (N)  Low (L)  Borderline (BL)  High (H)





Anemia Profile

Anemia is the condition where your body has less RBCs (red blood cells) or the RBCs don't have enough haemoglobin. Haemoglobin is the protein present in RBCs that help carry oxygen to your body's tissues.

Hemoglobin: 13.6 g/dL

● NORMAL



- Hemoglobin is present in the Red Blood Cells and it carries oxygen to the tissues
- If Hb is less it causes anemia
- Anemia because of low hemoglobin and is more common in women

Abnormal results may indicate :



Anemia.

Diet and Lifestyle Tips :



Eat iron rich foods as iron is essential for the production of hemoglobin. Iron-rich foods include meat, fish, eggs and oysters, beans, lentils, dark green leafy vegetables (spinach, watercress, curly kale), broccoli, iron fortified cereals and dried fruits (apricots, prunes and raisins).



Avoid drinking tea and coffee with meals, and foods with high phytic acid, such as whole grain cereals, as they can affect digestive absorption of iron from your diet.



Your body absorbs iron from plant-based foods better when you eat them with vitamin-C rich foods, such as oranges, strawberries, melons, peppers and tomatoes.





Blood Counts

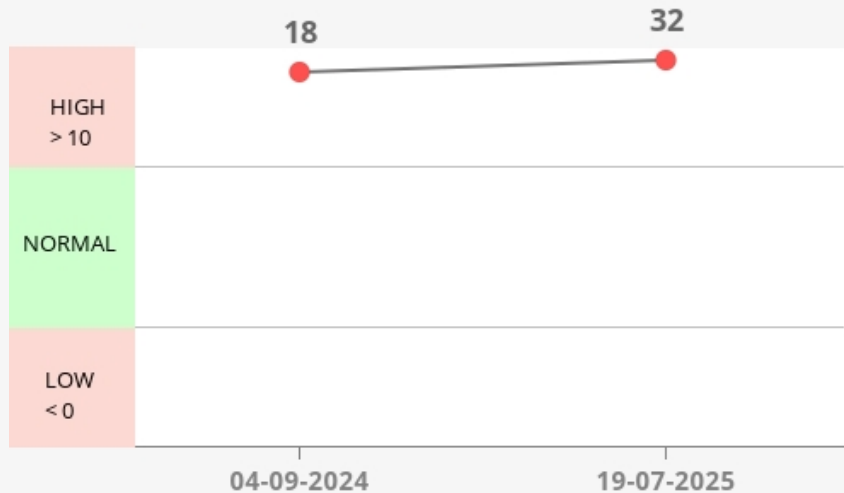
Blood is a specialized bodily fluid that supplies essential substances like sugars, oxygen, hormones - around the body and also removes waste from the cells.

Solid part of your blood (roughly 45%): RBCs (red blood cells), WBCs (white blood cells) and platelets

Liquid part of your blood (roughly 55%, usually called plasma): Water, Salts and Proteins

ESR - Erythrocyte Sedimentation Rate: 32 mm/hr

● HIGH



- ESR (Erythrocyte Sedimentation Rate) is the speed at which RBCs in your sample settle down at the bottom of the test tube
- If there is inflammation in the body then ESR value is high

Abnormal results may indicate :



Infections



Renal disease



Anemia



Acute allergy



Pregnancy (but not first trimester)



Old Age



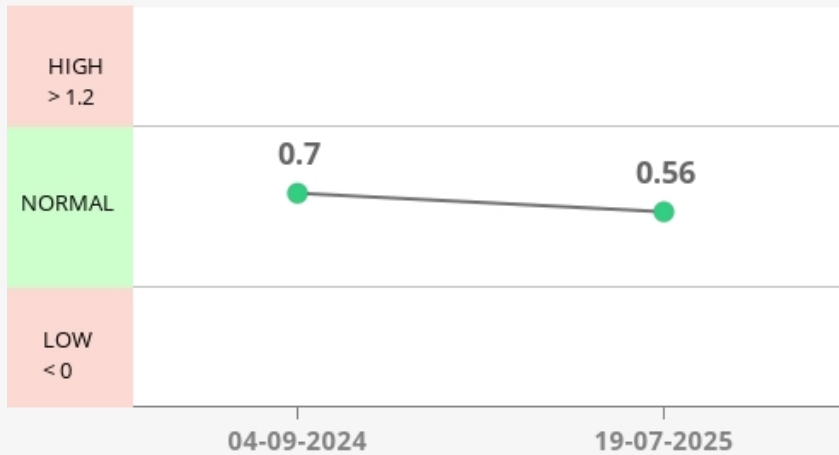


Liver Profile

One of the main functions of your liver is to make proteins that are secreted in your blood. It also makes enzymes which convert food into energy, and processes old muscles and cells. When your liver is damaged, enzymes leak into your blood and appear in the blood test

Bilirubin Total: 0.56 mg/dL

● NORMAL



○ Bilirubin is released as a breakdown product formed by the liver from the hemoglobin of old RBCs

Common reasons for abnormal results :



Presence of Gallbladder stones



Liver Disorders like Hepatitis- Inflammation of the liver, Alcoholic liver disease



Reaction to drugs



Increased destruction of Red Blood Cells

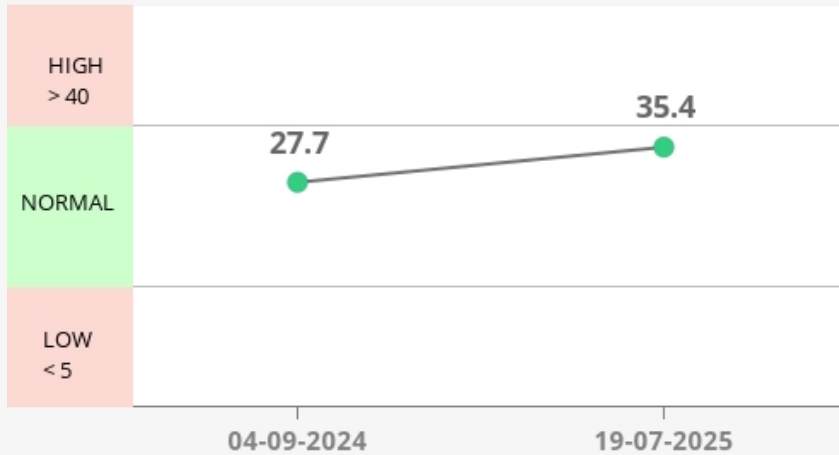
Enzymes

Enzymes found in your liver are responsible for various processes that maintain body functions. These enzymes are leaked into your blood when your liver suffers dysfunction.



SGOT/AST: 35.4 U/L

● NORMAL



- AST is an enzyme your liver makes
- Other organs, like your heart, kidneys, brain, and muscles, also make smaller amounts
- AST is also called SGOT (serum glutamic-oxaloacetic transaminase)
- Normally, AST levels in your blood are low
- When your liver is damaged, it puts more AST into your blood, and your levels rise

Common reasons for abnormal results :



Intense exercise, muscle injury, polymyositis (inflammatory disease of muscles), hypothyroidism, acute myocardial infarction and pancreatitis can also increase the level of AST and ALT in your blood.



Obesity, insulin resistance and type-2 diabetes increases your risk of developing (NAFLD - non-alcoholic fatty liver disease). Mild and fluctuating elevation of AST and ALT is seen in people with NAFLD.



Drugs such as aspirin, acetaminophen, anti-tuberculosis drugs and statin can cause mild increase in your AST and ALT levels.

Abnormal results may indicate :

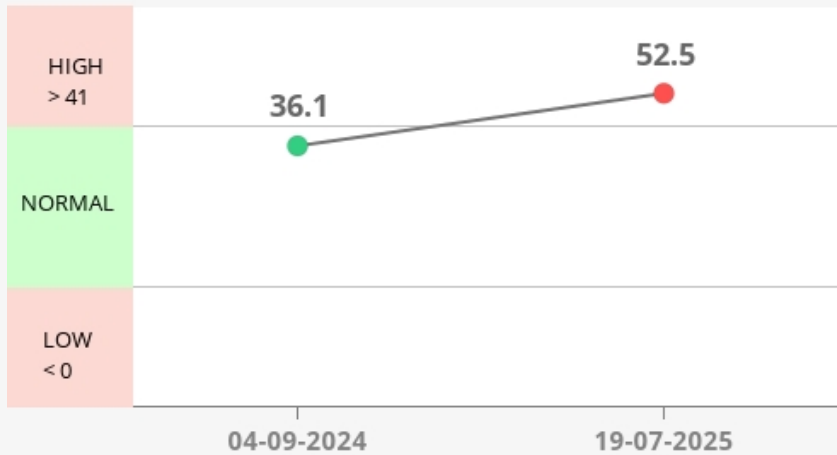


If both AST and ALT are increased, it may indicate liver dysfunction. If only AST is elevated while ALT is normal, dysfunction in other organs needs to be ruled out.



SGPT/ALT: 52.5 U/L

● HIGH



- SGPT is mostly concentrated in your liver and is a vital indicator of your liver's health
- It is also called alanine aminotransferase
- Serum ALT level, serum AST (aspartate transaminase) level, and their ratio (AST/ALT ratio) are commonly measured as biomarkers for liver health

Common reasons for abnormal results :



Intense exercise, muscle injury, polymyositis (inflammatory disease of muscles), hypothyroidism, acute myocardial infarction and pancreatitis can also increase the level of AST and ALT in your blood.



Obesity, insulin resistance and type-2 diabetes increases your risk of developing (NAFLD - non-alcoholic fatty liver disease). Mild and fluctuating elevation of AST and ALT is seen in people with NAFLD.



Drugs such as aspirin, acetaminophen, anti-tuberculosis drugs and statin can cause mild increase in your AST and ALT levels.

Abnormal results may indicate :



If both AST and ALT are increased, it may indicate liver dysfunction. If only AST is elevated while ALT is normal, dysfunction in other organs needs to be ruled out.



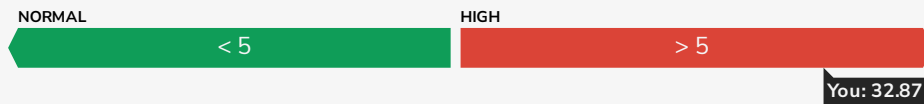
Inflammation

Inflammation is the body's immune system's response to an injury, surgery, or irritation. This natural defense process acts by removing injurious stimuli and initiating the healing process. Inflammation can be chronic (such as arthritis) or acute (like in case of trauma).

CRP (Quantitative): 32.87 mg/L

● HIGH

CRP (C-reactive protein) is a very sensitive test for early diagnosis of inflammation. Levels of CRP rise rapidly after inflammation. CRP levels are also used to monitor effectiveness of treatment in a particular patient. This test can also give an estimation of the degree of inflammation.



Did You Know?



CRP values are typically higher in women and elderly.



Temporary, very high levels of CRP are most commonly associated with infection.



The level of CRP increases when there is inflammation in the body. CRP may be used to diagnose sepsis, arthritis, autoimmune diseases, and chronic diseases among others.



In many studies, muscle strengthening exercise, especially in women, has shown to decrease CRP.



Obesity, insomnia, depression, smoking, and diabetes can all contribute to mild elevations in CRP. Results shall be interpreted with caution in such cases.



In diseases like rheumatoid arthritis CRP levels remain persistently high.



About Redcliffe Labs

We are India's Most Trusted & Fastest Growing Network of Diagnostics Labs

Best Customer Experience



Commitment to excellence, high end technology oriented staff

100% Report Correctness



Focus on quality with accurate results

Best Prices With Fast Reports



Value for money with quick turn around time (TAT)

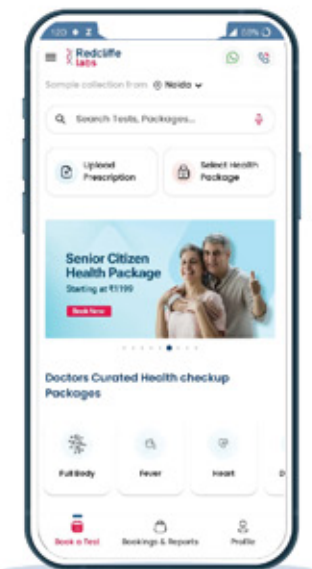
Your booking just gave back to nature –
with every health checkup, you're contributing by planting a tree!



Select from a wide range of
Full Body Health Check-up
packages that suits your health

Best Selling Package

**Annual Health Checkup
Advance Plus with Free HsCRP**



Get free report consultation
& download our app

A Simple and easy way to track your health

3600 + Tests & Packages



Presence in 220+ Cities



45 Min Express Sample Collection



100% Accurate Report Guarantee.

